



HealthCare

CHANDLER HOSPITAL
800 Rose St
Lexington KY 40536-
0536
Notes Report

MRN: 110834266, Legal
Sex: M
Adm: 3/18/2025, D/C: 4/21/2025

MRN: 110834266



Isler, Alexandra N, APRN, DNP
Nurse Practitioner
Oncology

Progress Notes  
Signed

Date of Service: 4/15/2025 12:13 PM

Medical Oncology Consult Follow-Up Note

04/15/25

Patient Care Team:

Pcp, No as PCP - General (Family Medicine)

Subjective:

Pt resting comfortably in bed. Sister at bedside. Today is D3 etoposide. States he developed an erythematous, warm chest yesterday during treatment (however, this was previously noted on AM PE). Proceeded at slower rate, without difficulties. Endorses ongoing shortness of breath. Denies chest pain. Good PO intake, without nausea or vomiting. Ongoing fatigue and weakness. Denies fevers, chills, headaches, diarrhea, constipation, difficulties voiding, or numbness or tingling of hands/feet.

PMH, Surg History, Social History, Family History and current medications reviewed and unchanged from last encounter of 4/14/25, or updated as indicated.

ROS:

Targeted review of systems completed and negative unless stated above.

Objective:

Visit Vitals

BP	118/59
Pulse	70
Temp	36.4 °C (97.6 °F)
SpO2	90%

ECOG: 2

General: Lying/resting comfortably in bed, NAD, fatigued/ill appearing, drowsy

HEENT: NCAT, PERRLA/EOMI, anicteric

Neck: Supple

Heart: RRR, no MGR

Lungs: Scattered expiratory wheezes on auscultation, no rales or rhonchi, 3L NC

Abdomen: Soft, NTND, + BS

Extremities: No edema

Musculoskeletal: No focal tenderness or deformity

Skin: Erythema of upper chest, warm to touch; no visible lesions

Neuro: Generalized weakness otherwise grossly nonfocal; no localizing deficits of strength, sensation, or mentation; although delayed responses

Psychiatric: Normal mood and thought content

Labs:

Labs in last 18 hours

CBC		
WBC 8.07	Hb 7.9 (L)	Plt 215
	Hct 26.0 (L)	

ANC 7.57 (H)

INR Ø, PTT Ø, Anti-Xa Ø

BMP			
Na 136; 136	Cl 108 (H); 108 (H)	BUN 14; 14	Glu 240 (H); 240 (H)
K 5.3 (H); 5.3 (H)	Co2 18 (L); 18 (L)	Cr 0.64 (L); 0.64 (L)	

Ca 7.8 (L); 7.8 (L) iCa 4.3 (L)

Mg 1.6 (L), Phos 2.0 (L)

Lactate Ø

LFT		
AST 39	AlkPhos 104	T Prot 5.6 (L)
ALK 68 (H)	Bili 0.3	Alb Ø

D.Bili Ø

Labs personally reviewed

Imaging: As noted in HPI if relevant.

Assessment/Plan:

66 y.o. male with history of newly diagnosed non small cell carcinoma with choriocarcinomatous differentiation with metastases to the mediastinal lymph nodes, bilateral adrenals, liver, and brain who was admitted for AMS. Medical oncology consulted for oncology plan of care.

Non-small cell carcinoma with choriocarcinomatous differentiation

RUL mass

Metastases to mediastinal nodes, bilateral adrenals, liver, brain (s/p crani)

- Will need to establish care with medical oncology (Corbin v UK)
- Diagnosed March 2025

- Current regimen: carboplatin + etoposide
- 3/18: transferred to UK from OSH
- S/p R occipital, R frontal crani for tumor resection (3/19); complicated by R hemiparesis
- Path + metastatic poorly differentiated non-small cell carcinoma, w squamoid features and markedly pleomorphic tumor cells; PD-L1 TPS >95%, HER2 3+, TP53 mutation, TMB high
- CARIS consistent with germ cell tumor (92%); elevated LDH and beta HCG (>94,000) support this diagnosis (see note from 4/12/25 for Caris report or Misc Path report in Lab tab)
- 4/12: testicular US unremarkable
- Results previously reviewed with pathology, likely represents a rare variant of non small cell carcinoma with choriocarcinomatous differentiation rather than a true GCT
- Continue dex, keppra
- Labs and PS stable, OK to proceed with C1D3 etoposide today

Cancer related pain

- Continue acetaminophen, gabapentin, lidocaine patch, oxycodone
- Palliative consulted for assistance with pain management

Gr 2 debility

- Likely multifactorial r/t non-sm cell carcinoma w choriocarcinomatous differentiation + anorexia
- Activity as tolerated
- PT/OT following
- Subacute reccs

Protein kcal malnutrition

Gr 2 anorexia

- Dietary following
- PO intake as tolerated/prescribed
- High calorie/high protein meals + snacks
- Continue Boost supplementation
- Closely monitor PO intake, calorie counts, I&Os

Gr 3 anemia

- No s/s acute blood loss
- Continue to monitor with CBC
- Transfuse if hgb /< 7.0

Plan 4/15/25:

- Labs and PS stable, OK to proceed with C1D3 etoposide today
- Closely monitor for adverse effects of chemotherapy, including CBC w diff for -cytopenias
- Continue allopurinol for TLS prophylaxis (started 4/12); moderate risk of TLS due to tumor size and high response rates to chemotherapy
- Await palliative care recommendations for cancer related pain
- FU with radiation oncology in Corbin, 4/21, for consolidation of brain tumor resection, GKRS preferred for limited tumor burden
- Will need to establish care with medical oncology (Corbin v UK); dependent on long term housing plan, sister v facility
- Dispo: SACU

- Rest per primary team

Medical oncology will continue to follow. Please call with additional questions or concerns.

> 40 minutes was spent on this encounter; including preparing to see the patient, which involved review/interpretation of diagnostics and reports; obtaining and/or reviewing separately obtained history; performing appropriate physical exam; ordering/scheduling medications, tests or procedures; communicating findings and counseling/educating the patient, family and/or caregiver; documentation in EMR; and care coordination.

The selection, dosing and administration of anti-cancer agents and the management of associated toxicities requires complex medical decision making and intensive monitoring for toxicity. Modifications of drug dose and schedule as well as the initiation of supportive care interventions are often necessary because of expected toxicities. This varies individually based on patient tolerability, prior treatments and comorbidities/risk status. Monitoring typically entails labs, imaging and/or other diagnostics, utilizing a healthcare delivery team experienced in the use of anticancer agents and the management of associated toxicities in patients with cancer.

Attending physician previously developed plan of care, which I discussed with the patient, whom I saw independently. The patient verbalized understanding and agrees with plan. All questions answered to their satisfaction. Encouraged to call should other questions/concerns arise.

Alexandra Isler, DNP, APRN, FNP-C
Medical Oncology

Electronically signed by Isler, Alexandra N, APRN, DNP at 4/15/2025 3:30 PM

ED to Hosp-Admission (Discharged) on 3/18/2025 *Note shared with patient*

Care Timeline

03/20 1813 Admitted from ED 1813
04/21 1715 Discharged 1715